

ESTIMATING THE EFFECT OF *DOBBS* ON FEMALE LABOR SUPPLY

AN HONORS THESIS

SUBMITTED ON THE FIRST DAY OF MAY, 2026

TO THE DEPARTMENT OF ECONOMICS

IN PARTIAL FULFILLMENT OF THE REQUIREMENTS

OF THE HONORS PROGRAM

OF NEWCOMB-TULANE COLLEGE

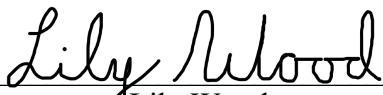
TULANE UNIVERSITY

FOR THE DEGREE OF

BACHELOR OF SCIENCE

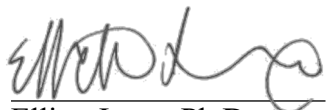
WITH HONORS IN ECONOMICS

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Lily Wood. Estimating the Effect of *Dobbs* on Female Labor Supply.

(Professor Elliott Isaac, Economics)

In 1973, *Roe v. Wade* legalized abortion in the United States. Existing literature has determined that this policy shift led to reductions in fertility and improvements in women’s educational and career outcomes, including labor force participation. In 2022, *Dobbs v. Jackson Women’s Health Organization* overturned *Roe* and returned authority over abortion regulation to the states. After *Dobbs*, states with “trigger-laws” experienced a reversal in abortion access essentially overnight, creating the first meaningful variation in access in almost fifty years. If expanding access to abortion improved female labor force participation, will reversing access to abortion have the opposite effect?

Using microdata from the Current Population Survey’s Annual Social and Economic Supplement (ASEC), I employ a difference-in-differences design, measuring female labor force participation at the state level before and after *Dobbs*. Although similar studies post-*Roe* found an effect on female labor force participation, I find no statistically significant impact. These findings hold across a range of robustness checks and an alternative specification using a continuous measure of gestational limits.

It is possible that the labor market effects of abortion policy take longer to appear than the treatment period presented by this thesis, or that the effects that do exist are along the margins—hours worked, for example. It is also possible, however, that the relationship between abortion policy and female labor supply has fundamentally changed since 1973, and a woman’s ability to avoid or delay unintended pregnancy is no longer as closely tied to her state’s abortion policy or her labor force participation.

Acknowledgements

First and foremost, I would like to thank my thesis director, Professor Elliott Isaac, for his patience, guidance, and thoughtful feedback at every step along the way.

I am grateful to my second reader, Professor Katy Bergstrom, for her insights and for her course in development economics, which introduced me to empirical economics and inspired me to conduct my own research.

Lastly, I am endlessly grateful to my parents for their unconditional love, support, and encouragement of my education.

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I. Introduction

If access to abortion reduces fertility, and if fertility reduces female labor supply, then bans on abortion should, in theory, raise fertility and therefore reduce female labor force participation. This hypothesis rests on two well-established relationships: the effect of fertility on women's labor supply and the effect of abortion access on fertility.

The literature demonstrates that childbearing creates substantial and persistent reductions in women's labor market attachment, earnings, and hours. This gap is often referred to as the "child penalty," and applies across low-, middle-, and high-income countries. The strongest negative effects, however, emerge in highly developed economies where women face steeper opportunity costs of leaving the labor market (Bloom et al. 2009, Kleven et al. 2019, Aaronson et al. 2020).

In the United States, empirical evidence demonstrates that motherhood sharply reduces women's labor force participation in both the short and long run. The birth of a first child reduces female labor force participation by 26 percentage points in the first year (Cristia 2008), and the gap is persistent: 24 percent of new mothers exit the labor force or shift to part-time in the first year (Lu et al. 2017), 21 percent of mothers remain out of the labor force for the first 18 years (Killewald & Zhuo 2019), and six years after a first birth, women are 26 percentage points less likely to be in the labor force than they were one year before the birth (Sandler and Szembrot 2019).

Considering this negative relationship, economists have long studied the policies that allow women to control the timing of motherhood: contraception and abortion. Research using evidence from the *Roe* era estimates that the introduction of legal abortion lowered birthrates by between 5 and 13 percent, especially among teenagers, older women, and

women of color (Levine et al. 1996; Ananat et al. 2007; Guldi 2008). A key question is whether this relationship still holds in the modern United States—and research suggests that it does. Between 2011 and 2014, Texas enacted a series of restrictions on abortion clinics, causing some counties to experience closures. In those counties, abortion fell by roughly 17 percent and births increased by 1–2 percent (Fischer et al. 2018). Notably, early evidence from the post-*Dobbs* period points to a similar pattern: Dench et al. (2024) find that states enforcing near-total abortion bans in 2023 experienced a 2.3 percent increase in births.

At the same time, the diffusion of oral contraception in the late 1960s and early 1970s played an important role in enabling women to delay fertility, invest in education, and enter the labor force (Goldin & Katz 2002; Bailey 2006; Bailey et al. 2012). This raises the question: to what extent were improvements in women’s economic outcomes driven by the pill rather than by the legalization of abortion? Ananat and Hungerman (2012) find that the two served different purposes and were not close substitutes. Meyers (2017) concludes that the legalization of abortion, rather than access to the pill, allowed many young women to delay marriage and motherhood and led to larger declines in unintended fertility.

Having established that fertility reduces female labor supply and that access to abortion reduces fertility, a natural question follows: does abortion access have a direct effect on female labor supply? International evidence suggests that, following the liberalization of abortion policy, women tend to delay childbearing and maintain stronger labor market attachment (Mølland 2016; Adda et al. 2017; Lundborg et al. 2017; Kleven et al. 2019). In the United States, evidence from before *Roe v. Wade* confirms this

relationship. Prior to the ruling, a handful of states had already legalized abortion. Kalist (2004) finds that women in these states were roughly 2 percentage points more likely to work 40 or more weeks per year. Additionally, Angrist and Evans (1996) find that liberalization reduced teenage fertility and, in turn, increased high school completion, college attendance, and employment for young Black women.

Against this backdrop, *Dobbs v. Jackson Women's Health Organization* represents a unique opportunity to revisit a classic economic question: to what extent does abortion access affect female labor force participation? Early research has begun to document fertility responses to post-*Dobbs* abortion bans (Dench et al. 2024), but empirical evidence on the economic effects of the decision, including labor market effects, remains limited due to its recency. This thesis adds to the literature by providing early empirical evidence on the short-run labor market effects of post-*Dobbs* abortion bans. I leverage the sharp divergence in abortion access between states following the decision to compare changes in female labor force participation between states that enacted immediate bans and those that did not.

II. Policy Background

Prior to 1973, abortion was essentially illegal nationwide under state restrictions and prohibitions. Although a handful of states legalized the procedure early, that was the exception rather than the rule. With the landmark Supreme Court case *Roe v. Wade*, abortion became a constitutionally protected right in the United States. The ruling remained in effect for almost fifty years, ensuring that—despite attempts by more conservative states to restrict access to the procedure—abortion remained legal until viability across the board.

Even under *Roe v. Wade*, states imposed a variety of restrictions on abortion clinics to limit access to the procedure. Mandatory waiting periods, parental consent and notification laws, and Targeted Regulation of Abortion Providers (TRAP)—laws that require abortion clinics to be held to the same standards as a hospital or surgical center—led to delayed or forgone abortions, a lower quality of care, and clinic closures. These restrictions disproportionately impacted young and low-income women (Myers 2021; Myers & Ladd 2020; Lindo et al. 2021; Lindo & Pineda-Torres 2024) and even reduced college enrollment and completion among affected young women (Jones & Pineda-Torres 2024).

In 2018, Mississippi introduced the “Gestational Age Act,” essentially a 15-week ban on abortion. This policy conflicted with *Roe v. Wade*, which protected abortion until fetal viability—between 22 and 24 weeks. Mississippi’s sole abortion clinic challenged the statute, and the case eventually reached the Supreme Court in *Dobbs v. Jackson Women’s Health Organization*.

In May 2022, a draft of the majority opinion was leaked to the press. In a surprising and unprecedented development, it became clear that the Court was preparing not only to uphold the statute but to overturn *Roe*. On June 24, 2022, the Court issued its final decision, ruling in favor of Mississippi and formally overturning *Roe v. Wade*, returning full authority over abortion regulation to the states.

Even before *Dobbs*, conservative states had been pushing to restrict abortion—so much so that a subset of states had “trigger laws” on the books at the time of the decision, essentially teeing up abortion bans to go into effect if the federal barrier were ever reversed. When *Dobbs* did just that, abortion became illegal in thirteen states. Other states passed strict gestational limits, mandatory waiting periods and ultrasounds, or parental consent and notification laws.

As a result of *Dobbs v. Jackson Women’s Health Organization* and subsequent “trigger laws” and abortion bans, a policy environment has emerged in which access to abortion varies dramatically by state—from legally protected throughout pregnancy in some to prohibited without exception in others. Notably, the policy break created by *Dobbs* was unique. The reversal of the landmark *Roe v. Wade* creates the first true variation in access and plausibly exogenous “shock” to abortion policy since the 1970s.

III. Empirical Strategy

To estimate the causal effect of access to abortion on female labor supply, I use *Dobbs v. Jackson Women's Health Organization* as an exogenous shock. Following the decision, thirteen states with “trigger laws” experienced immediate abortion bans, creating a natural treatment and control group. I estimate the effect using a difference-in-differences design that compares changes in female labor force participation before and after *Dobbs* between states that implemented immediate abortion bans and states that did not.

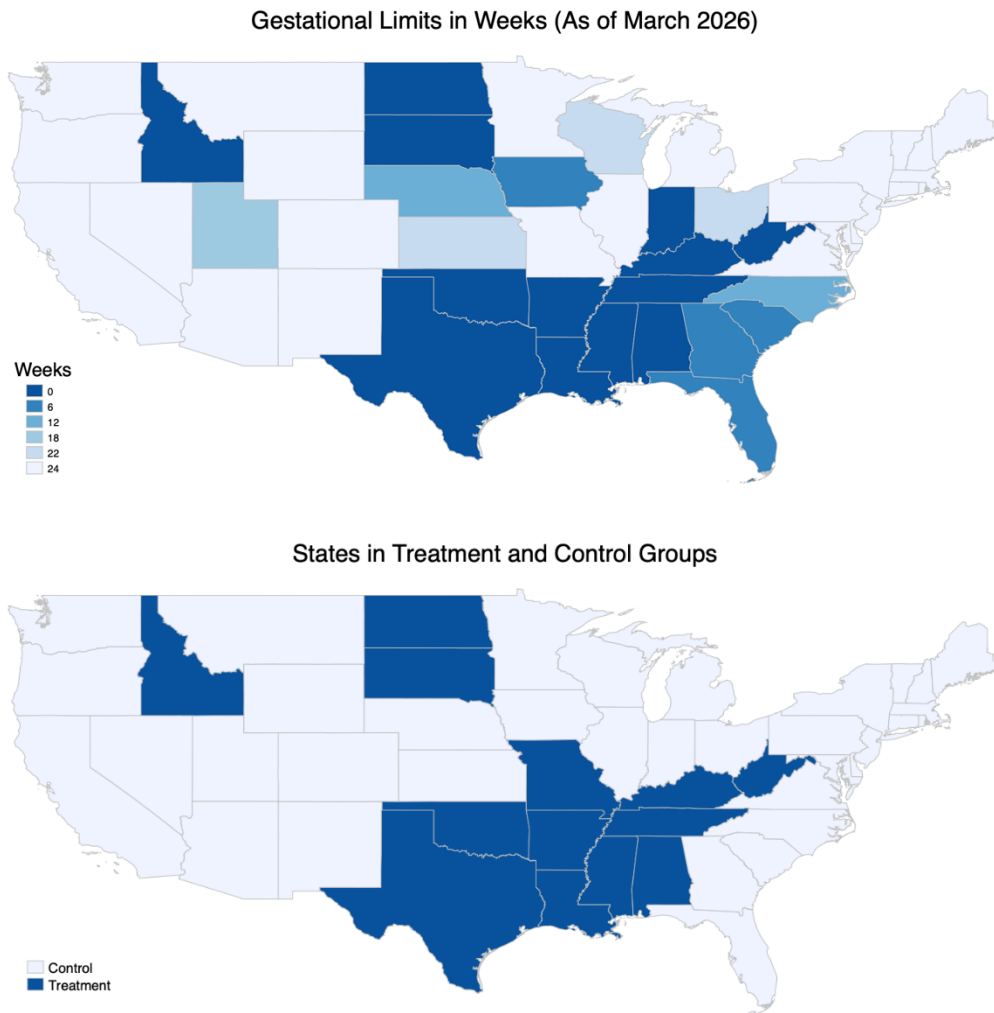
A. Treatment and Control

I define the treatment group as states with “trigger laws” that implemented total abortion bans in the immediate aftermath of *Dobbs*. In this context, a “total” ban is defined as a ban on abortion at any point during pregnancy. In other words, states with strict gestational limits—such as 6-, 12-, or 15-week bans—were not included in the treatment group for the difference-in-differences analysis. Instead, I capture the effect of gestational limits with an alternative specification.

I have defined the “immediate aftermath” as between the *Dobbs* decision on June 24, 2022 and September 13, 2022. This cut-off date allows for the inclusion of West Virginia, the last state to implement its trigger law. A few of the “trigger laws” required attorney general certification or a 30-day waiting period before enforcement, hence the buffer in the treatment group, but in all cases the laws were activated by *Dobbs* itself. Following these criteria, the treatment group consists of the following thirteen states: Alabama, Arkansas, Idaho, Kentucky, Louisiana, Mississippi, Missouri, North Dakota, Oklahoma, South Dakota, Tennessee, Texas, and West Virginia.

I define the control group as the states that did not implement a total abortion ban for longer than seven days during the immediate aftermath of *Dobbs*. A grace period of seven days allows for the inclusion of Utah and Wyoming, whose trigger laws technically took effect immediately after *Dobbs* but were quickly blocked by courts. Following these criteria, the control group consists of the remaining thirty-seven states.

Figure 1. Current Gestational Limits & Treatment and Control Groups



Notes: Data from the Guttmacher Institute & the Center for Reproductive Rights.

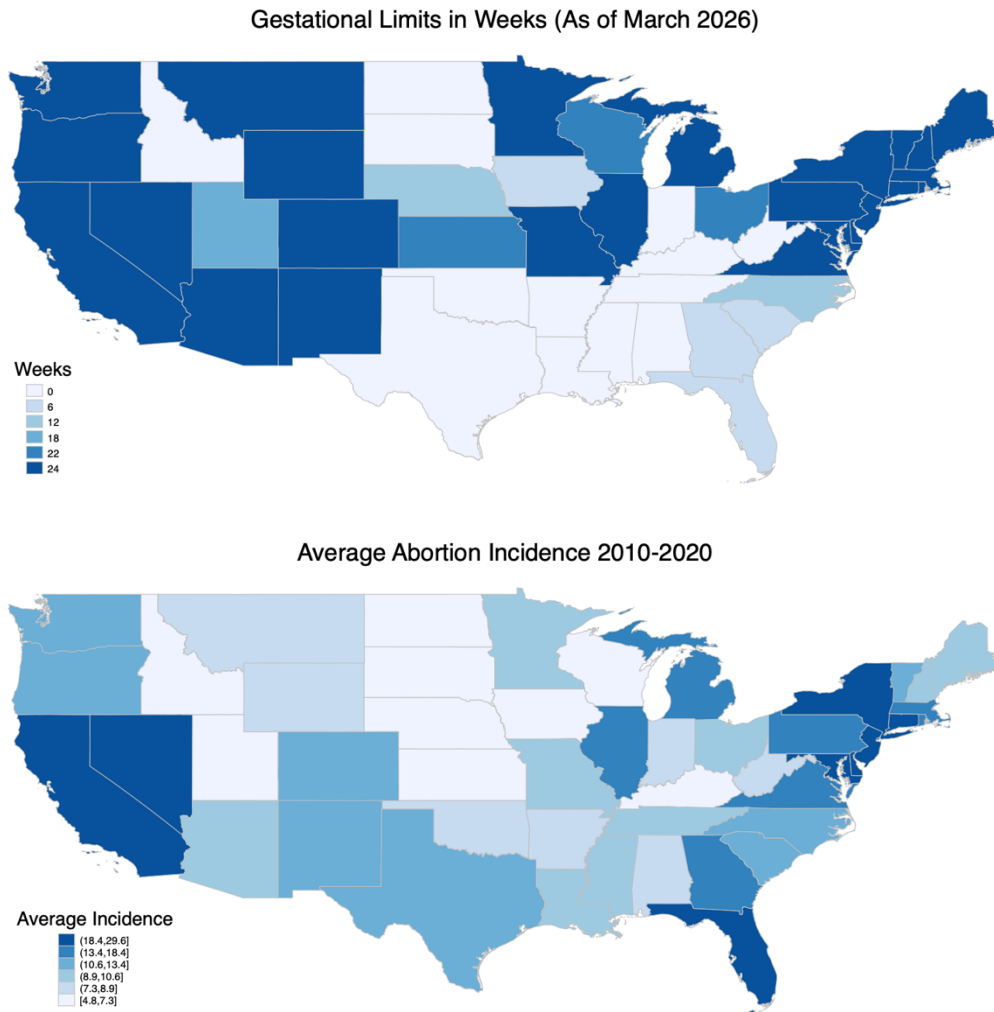
As illustrated by Figure 1, much of the state-level variation in gestational limits following *Dobbs* is not captured by the definition of “treatment” for the primary

specification. The treatment group only includes states with an effective gestational limit of zero and excludes other states with highly restrictive policies that may also affect access to abortion, and its subsequent labor market outcomes. Notably, Georgia, Florida, Iowa, and South Carolina all restrict abortion after six weeks of gestational age, before many women even know they are pregnant. This raises the question of how to account for this variation.

The difference-in-differences design relies on a binary treatment variable that changes discretely with *Dobbs*. To isolate such a variable, I selected the trigger law states for the primary specification—in these states, the lack of access to abortion was total and immediate, varying only before and after *Dobbs*. Gestational limits, by contrast, have varied within the post-treatment period and are continuous in nature—a 6-week ban is dramatically different from a 15-week ban. For this reason, their impact is better captured by a regression on a continuous measure of gestational limits that varies by state and year. This alternative specification includes all states, from those with a gestational limit of zero to those with limits as high as forty weeks.

As seen in Figure 1, the treatment states are concentrated in the southern United States. This raises a second question: are the cultural and political attitudes regarding abortion too different between the treatment and control groups? While it is true that conservative states tend to have stricter abortion policies, it is not necessarily true that the women living in those states are all the same. Although the policy variation is at the state level, the analysis uses microdata, so the assignment of women to treatment and control groups is far closer to random than the assignment of abortion policy to states.

Figure 2. Current Gestational Limits & Pre-*Dobbs* Abortion Incidence

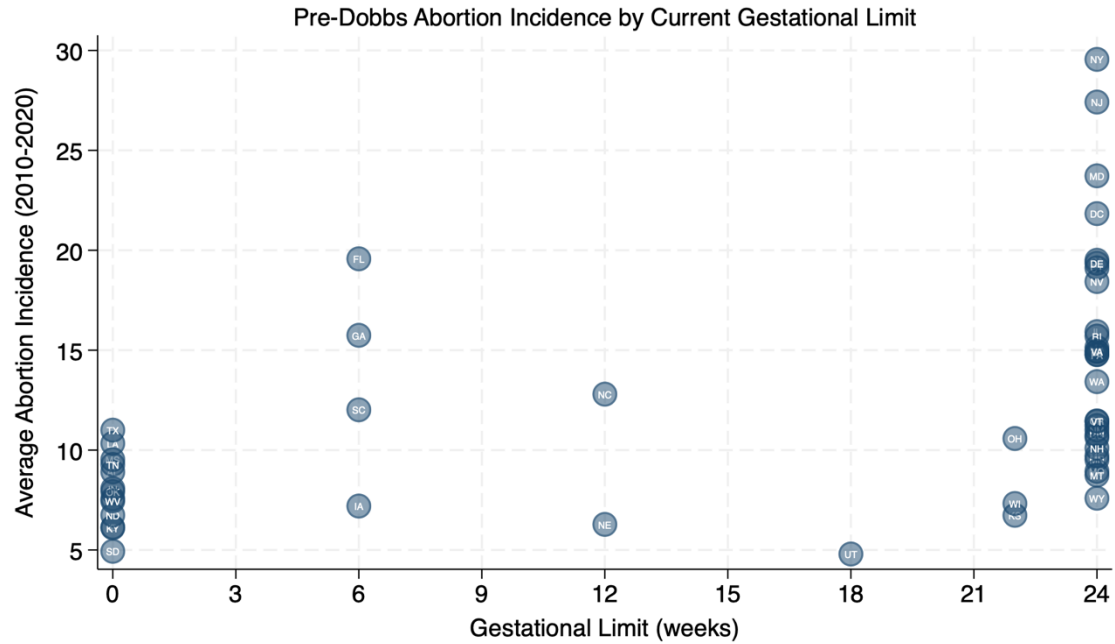


Notes: Data from the Guttmacher Institute & the Center for Reproductive Rights. Abortion incidence is defined as the number of abortions per 1,000 women age 15–44.

Additionally, although abortion policy often follows regional or partisan lines, the pre-*Dobbs* abortion incidence does not neatly align with the treatment and control groups. As illustrated by Figure 2, some southern states recorded abortion rates comparable to or higher than those in the Northeast, and even neighboring states sometimes differed by a factor of two. Oklahoma, a treatment state, had a pre-*Dobbs* abortion incidence that was more than double that of Utah, a control state. Further, the map of pre-*Dobbs* abortion

incidence does not neatly align with the map of current gestational limits, which I use as a proxy for a state's abortion restrictiveness.

Figure 3. Pre-*Dobbs* Abortion Incidence by Current Gestational Limit



Notes: Data from the Guttmacher Institute. Abortion incidence is defined as the number of abortions per 1,000 women age 15–44.

Figure 3 further illustrates this relationship and highlights a few outliers: Florida, Georgia, and South Carolina stand out as states with low gestational limits but high pre-*Dobbs* abortion incidence; Utah and Wyoming are examples of states with relatively liberal abortion policy but very low abortion incidence. In summary, it would be incorrect to assume that cultural attitudes or the political leanings of a state directly determine how many women obtain abortions. Even in states that were conservative enough to cut off access, there were women actively relying on that access prior to *Dobbs*.

B. Identification

Under this framework, the identifying assumption is that, in the absence of *Dobbs*, female labor force participation would have changed at the same rate in both groups. To identify the effect, I will compare the change in female labor force participation rates over time between treatment and control states using the following regression:

$$FLFPR_{ist} = \alpha + \lambda(Total_{Ban_s} \times Post_t) + X_{ist} + \mu_s + \tau_t + \varepsilon_{ist}$$

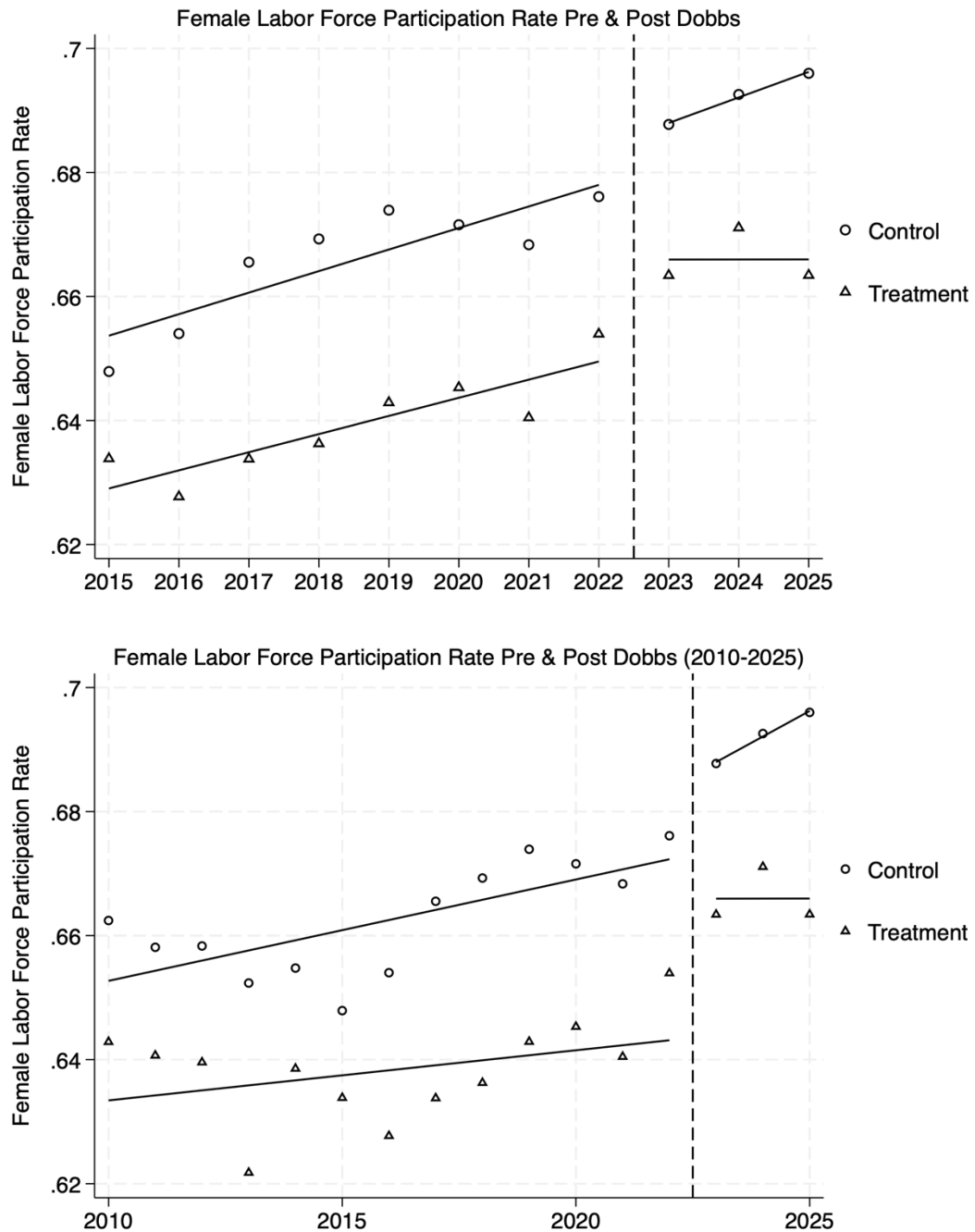
I selected female labor force participation as the outcome variable to focus on the decision of a woman to work rather than her employment—being “in the labor force” captures all women who intend to work, regardless of whether they currently have a job. The interaction term, $Total_Ban_s \times Post_t$ is an indicator for living in a treatment state after *Dobbs* and equals zero otherwise; in other words, it identifies the women we expect to be experiencing the effect of the policy change. The coefficient λ represents the percentage point change in female labor force participation in treatment states after *Dobbs* relative to the change in control states over the same period.

The control variables include standard demographics such as age, race, education, and poverty, as well as factors particularly relevant for fertility, including marital status, number of children, and number of children under five. They do not include variables related to work or income, as these are too closely correlated with the outcome variable. State fixed effects control for characteristics of a state that do not change over time—for example, South Dakota is far less populated than Texas, and this did not change over the study period. Time fixed effects control for shocks that affected all states equally at a given point in time, which is particularly relevant given that the Great Recession and

COVID-19 almost certainly had an impact on my outcome variable, labor force participation.

C. Parallel Trends

Figure 4. Female Labor Force Participation Rate Pre & Post *Dobbs*



Notes: The first graph illustrates trends from 2015–2025; the second illustrates trends for the full pre-treatment period, 2010–2015.

The parallel trends assumption is necessary for the difference-in-differences identification to hold. This assumption states that, absent the policy variation, female labor force participation in treatment states would have continued to grow at the same rate as in control states. Visually, the trends in female labor force participation rates appear to satisfy the parallel trends assumption in the pre-treatment period, both from 2015–2022 and, more weakly, from 2010–2022. Additionally, the raw data provides some indication of a break in trend: in both graphs, the FLFPR appears to flatten in treated states after *Dobbs*, while continuing to rise in control states, suggesting some motivation for further analysis.

IV. Data

I use microdata from the Current Population Survey's Annual Social and Economic Supplement (ASEC). The ASEC includes identifiers for state of residence and survey year, which allow me to construct state-by-year measures of female labor force participation in both treatment and control states.

Following the literature, I restrict the sample to women between the ages of 15 and 44. Across almost all research and data collection on reproductive health, this is the standard age range. It is a concern that labor market outcomes may differ for younger women, particularly those ages 15–22, whose labor market decisions may be influenced by schooling. However, my findings are robust across the full age range of 15–44, as well as more specific ranges of 18–24 and 25–44.

I define the study period as 2010–2025. In setting 2010 as the lower bound, I attempted to avoid distortions from the 2008–2009 Great Recession while balancing the distortion from the 2020–2021 COVID-19 pandemic that would have arisen had I selected a shorter pre-treatment period. However, my findings were robust using both the 2010–2025 window and the 2015–2025 window.

Table 1. Baseline Summary Statistics

	Control	Treatment	Difference
Age	29.84	29.71	0.13
Race (%)			
White	75.56	78.30	-2.74
Black	11.95	14.52	-2.57
Native American	1.41	1.95	-0.54
Asian or Pacific Islander	8.38	3.21	5.16
Other	2.70	2.01	0.69
Education (%)			
Less Than High School	20.18	22.39	-2.21
High School	21.05	23.64	-2.59

Less Than College	28.47	29.54	-1.06
Bachelor's Degree	20.03	17.15	2.88
Graduate Degree	10.27	7.29	2.98
Married (%)	42.26	44.61	-2.35
Number of Children in Household	1.04	1.15	-0.11
Number of Children Under 5 in Household	0.30	0.32	-0.02
Below Poverty Line (%)	15.09	18.39	-3.30
Employed (%)	61.63	59.76	1.87
In the Labor Force (%)	66.16	63.80	2.36

Note: Pre-treatment period defined as 2010–2022.

Table 1 presents baseline summary statistics for treatment and control groups during the pre-treatment period. Women in treatment states are, on average, slightly younger, more likely to be married, have more children, and are more likely to live below the poverty line. They also have lower levels of educational attainment and slightly lower baseline labor force participation rates. Given these differences, all demographic factors (i.e., excluding employment and labor force participation) were included as control variables in my regression. This table highlights the importance of the difference-in-differences design, which accounts for baseline differences between groups.

V. Results

As the primary specification, I estimate a difference-in-differences model of female labor force participation between states with and without trigger laws and subsequent abortion bans following the *Dobbs* decision. I find no statistically significant evidence of a change in female labor force participation, a conclusion that holds across a variety of robustness and heterogeneity checks, as well as a secondary specification utilizing gestational limits.

A. Trigger-Law Difference-in-Differences

Table 2. Difference-in-Differences Estimates

	(1)	(2)	(3)	(4)
Treatment	-0.024*** (0.002)	-0.024 (0.016)	-0.062*** (0.001)	-0.056*** (0.001)
Post	0.030*** (0.002)	0.030*** (0.005)	0.036*** (0.007)	-0.013*** (0.004)
Treatment $\times$ Post	-0.003 (0.004)	-0.003 (0.007)	-0.001 (0.006)	0.000 (0.005)
Observations	580130	580130	580130	580130
R-squared	0.001	0.001	0.009	0.213

Notes: Column (1): robust standard errors. Columns (2)–(4): standard errors clustered at the state level. Columns (3)–(4): include state and year fixed effects. Column (4): includes demographic controls.

The “Treatment” coefficient represents the difference in female labor force participation between treatment and control states before *Dobbs*. Using Column 4, the most comprehensive specification, female labor force participation was approximately 5.6 percentage points lower in treatment states relative to control states. This result indicates meaningful baseline differences between the two groups, which is consistent with Table 1.

The “Post” coefficient represents the change in female labor force participation in control states before and after *Dobbs*. Using Column 4, female labor force participation

in control states decreased by approximately 1.3 percentage points. This coefficient is statistically significant but varies across specifications and reflects broader trends in labor force participation rather than the effect of policy.

The “Treatment x Post” coefficient, which represents the difference-in-differences estimate, is not statistically significant in any specification—in fact, in the preferred specification it is 0.00. This result indicates no meaningful change in female labor force participation in treatment states relative to control states following *Dobbs*. Although the regression may contain some error, the estimates are sufficient to rule out large effects. In the preferred specification, the 95% confidence interval implies that the true coefficient lies between (-0.01, 0.01), meaning the true effect lies between -1 and +1 percentage point. Therefore, any effect of *Dobbs* on female labor force participation would be quite small.

B. Gestational Limits Regression

Table 3. Alternative Measure: Gestational Limits

	(1)	(2)	(3)	(4)
Abortion	0.00029	0.00101	-0.00063	-0.00009
Restrictiveness	(0.00222)	(0.00190)	(0.00163)	(0.00121)
Observations	367117	367117	367117	367117
R-squared	0.000	0.214	0.009	0.219

Notes: Column (1): robust standard errors. Columns (2)–(4): standard errors clustered at the state level. Columns (3)–(4): include state and year fixed effects. Column (4): includes demographic controls.

As a secondary specification, I constructed a continuous variable of gestational limits that varies by state and year. In practice, lower gestational limits correspond to more restrictive policies. To align the direction of this variable with the expected effect, I rescale gestational limits as:

$$Restrictiveness = \frac{24 - \text{gestational limit}}{6}$$

This transformation ensures that higher values correspond to more restrictive policies. For example, a state with a gestational limit of 24 weeks has a value of $(24 - 24) / 6 = 0$. An 18-week limit yields $(24 - 18) / 6 = 1$, a 12-week limit yields 2, and a 6-week ban yields 3. For states with total bans, an effective gestational limit of zero, the value is $(24 - 0) / 6 = 4$. Regressing female labor force participation on this measure, the coefficient can be interpreted as the percentage point change in participation associated with a six-week tightening of the gestational limit, or a one-unit increase in the restrictiveness measure.

Prior to *Dobbs*, when restricting access to abortion before fetal viability was illegal, all gestational limits are coded as 24 weeks. In the post-*Dobbs* period, although a handful of states do not have gestational limits, I cap the variable at 24 weeks. Because less than 1% of abortions occur after viability, this avoids placing numerical weight on variation that is unlikely to have a real-world effect.

I find that the coefficient on gestational limits is not statistically significant: a six-week reduction in the gestational limit is associated with a 0.03 percentage point decrease in female labor force participation. Even allowing for error, the 95% confidence interval rules out statistically significant effects. The true coefficient lies between -0.003 and 0.003 , implying that the effect is no larger than a 0.03 percentage point change.

C. Robustness Checks

I conduct a series of robustness checks to assess the sensitivity of the results to alternative sample restrictions and specifications, with coefficients presented in the appendix. As described above, these include restricting the sample to narrower age ranges (18–24 and 24–44), shortening the time window to 2015–2025, estimating the model using only Black women, and excluding North Dakota and Missouri from the treatment

group due to their more limited or uncertain enforcement. Across all specifications, the estimated effects remain small and statistically insignificant. The point estimates remain close to zero, and the confidence intervals continue to rule out meaningful effects, suggesting that the main results are not driven by these alternative specifications.

Of particular interest, I conduct a robustness check in which states with 6-week bans (Georgia, Florida, and South Carolina) are included in the treatment group. Women with unplanned pregnancies are among the least likely to know they are pregnant at six weeks. As a result, a 6-week ban may effectively function as a total ban for this group. For this reason, including these states in the treatment group is a useful robustness check for the empirical design.

Table 4. Alternative Treatment Definition: Including 6-Week Ban States

	(1)	(2)	(3)	(4)
Treatment	-0.017*** (0.001)	-0.017 (0.015)	-0.061*** (0.001)	-0.055*** (0.002)
Post	0.033*** (0.002)	0.033*** (0.005)	0.039*** (0.006)	-0.011*** (0.004)
Treatment $\times$ Post	-0.010*** (0.004)	-0.010 (0.007)	-0.008 (0.006)	-0.005 (0.005)
Observations	580130	580130	580130	580130
R-squared	0.001	0.001	0.009	0.213

Notes: Column (1): robust standard errors. Columns (2)–(4): standard errors clustered at the state level. Columns (3)–(4): include state and year fixed effects. Column (4): includes demographic controls.

This robustness check produces the results closest to statistical significance. In the most basic specification, having a gestational limit of six weeks or less is associated with a 1 percentage point reduction in female labor force participation. However, the estimate shrinks with the addition of fixed effects and demographic controls. While this robustness check points in the direction of the hypothesis, the estimates are not consistent across specifications and therefore do not support a causal interpretation.

VI. Discussion

Recalling findings on the effect of *Roe v. Wade* on female labor force participation raises the question of why, fifty years later, the reverse mechanism does not produce the predicted effect and instead yields a null result. It is possible that the labor market effects of abortion policy take longer to appear than the treatment period considered in this thesis. Returning to Figure 4, the break in parallel trends appears to be driven largely by 2025, the final year of the treatment period. Perhaps 2025 is an anomaly, or perhaps it is an indicator that the labor market effects of the *Dobbs* decision will be felt later down the line. If we look to the literature surrounding *Roe v. Wade*, Kalist (2004) finds labor market effects only after several years, suggesting that these responses may take time to materialize.

It is also possible that any labor market effects that do exist occur along the margins rather than through labor force participation itself. Previous work finds that fertility has a direct effect on other economic variables, such as earnings, hours worked, or transitioning to part-time. Although these outcomes are beyond the scope of this paper, they represent promising avenues for future research.

Even without *Roe v. Wade*, the landscape of reproductive healthcare and abortion access has changed dramatically since 1973, and a woman's ability to avoid or delay unintended pregnancy may no longer be as closely tied to her state's abortion policy. Following the *Dobbs* decision, a huge push expanded access to Mifepristone, a medication abortion that can be used until 10 weeks of pregnancy. Even in states without clinical abortion access, it is possible that demand is being met through prescriptions. Additionally, a limitation of this thesis is that it does not account for interstate travel.

Although traveling for abortion care is illegal in some states and impossible for many women due to financial or logistical barriers, the Guttmacher Institute has reported a dramatic increase in interstate travel for abortion care. As a result, even women living in treatment states may still be able to obtain abortion services.

Another important difference is that the *Roe v. Wade* decision in 1973 coincided with the widespread use and distribution of oral contraception. In 2026, far more than in 1973, abortion is not the only way—or even the primary way—for women to control the timing of motherhood. As a result, questions about how women’s ability to choose when to have children affects their participation in the labor force may now be less closely tied to abortion access and more closely tied to contraception. More broadly, abortion access itself may no longer be determined solely by the abortion policy of a woman’s state.

Finally, it is also possible that the relationship between fertility and labor force participation has changed. A key finding of earlier literature on fertility and female labor supply, such as Angrist and Evans (1996), is that mothers, rather than fathers, experience the labor market effects of additional children. While research suggests that this pattern still holds, the magnitude of these effects may be smaller than it was several decades ago. As a result, it is possible that abortion policy, while still an integral part of healthcare and women’s autonomy over their reproductive health, is less necessary than it once was to prevent women who experience unwanted pregnancies from leaving the labor force.

The policy shifts created by *Roe v. Wade* and *Dobbs v. Jackson Women’s Health Organization* are incredibly unique. The latter, in particular, provides an opportunity to examine empirically what has changed and what has remained the same regarding the economic effects of reproductive policy. The findings of this thesis suggest that quite a

bit may have changed, and perhaps for the better. Women should be able to choose how and when they participate in the labor force, regardless of the state in which they live, and it is a meaningful result that recent changes to reproductive policy appear to be less of a hindrance to that choice than those of the past.

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Appendix

Appendix A. Robustness Check: Ages 18–24

	(1)	(2)	(3)	(4)
Treatment	-0.016*** (0.005)	-0.016 (0.021)	-0.064*** (0.003)	-0.086*** (0.003)
Post	0.018*** (0.005)	0.018*** (0.006)	0.026** (0.010)	0.001 (0.010)
Treatment × Post	0.009 (0.010)	0.009 (0.012)	0.009 (0.012)	0.008 (0.012)
Observations	72947	72947	72947	72947
R-squared	0.000	0.000	0.013	0.113

Notes: Column (1): robust standard errors. Columns (2)–(4): standard errors clustered at the state level. Columns (3)–(4): include state and year fixed effects. Column (4): includes demographic controls.

Appendix B. Robustness Check: Ages 25–44

	(1)	(2)	(3)	(4)
Treatment	-0.024*** (0.002)	-0.024* (0.013)	-0.063*** (0.002)	-0.057*** (0.002)
Post	0.029*** (0.002)	0.029*** (0.005)	0.045*** (0.007)	0.010** (0.004)
Treatment × Post	-0.005 (0.005)	-0.005 (0.007)	-0.005 (0.007)	-0.004 (0.006)
Observations	252954	252954	252954	252954
R-squared	0.001	0.001	0.010	0.150

Notes: Column (1): robust standard errors. Columns (2)–(4): standard errors clustered at the state level. Columns (3)–(4): include state and year fixed effects. Column (4): includes demographic controls.

Appendix C. Robustness Check: Black Women

	(1)	(2)	(3)	(4)
Treatment	-0.008 (0.005)	-0.008 (0.011)	-0.049*** (0.003)	-0.024*** (0.004)
Post	0.019*** (0.006)	0.019** (0.009)	0.026** (0.012)	-0.018 (0.012)
Treatment × Post	0.000 (0.011)	0.000 (0.012)	-0.002 (0.012)	0.002 (0.011)
Observations	45325	45325	45325	45325
R-squared	0.000	0.000	0.005	0.243

Notes: Column (1): robust standard errors. Columns (2)–(4): standard errors clustered at the state level. Columns (3)–(4): include state and year fixed effects. Column (4): includes demographic controls.

Appendix D. Robustness Check: 2015–2025

	(1)	(2)	(3)	(4)
Treatment	-0.026*** (0.002)	-0.026* (0.014)	-0.070*** (0.001)	-0.062*** (0.002)
Post	0.027*** (0.002)	0.027*** (0.004)	0.044*** (0.006)	0.005 (0.004)
Treatment × Post	0.000 (0.004)	0.000 (0.006)	0.001 (0.006)	0.001 (0.005)
Observations	367117	367117	367117	367117
R-squared	0.001	0.001	0.009	0.219

Notes: Column (1): robust standard errors. Columns (2)–(4): standard errors clustered at the state level. Columns (3)–(4): include state and year fixed effects. Column (4): includes demographic controls.

Appendix E. Robustness Check: Excluding North Dakota & Missouri

	(1)	(2)	(3)	(4)
Treatment	-0.039*** (0.002)	-0.039*** (0.012)	-0.070*** (0.001)	-0.062*** (0.002)
Post	0.026*** (0.002)	0.026*** (0.004)	0.043*** (0.006)	0.005 (0.004)
Treatment × Post	0.004 (0.004)	0.004 (0.006)	0.004 (0.006)	0.002 (0.005)
Observations	367117	367117	367117	367117
R-squared	0.002	0.002	0.009	0.219

Notes: Column (1): robust standard errors. Columns (2)–(4): standard errors clustered at the state level. Columns (3)–(4): include state and year fixed effects. Column (4): includes demographic controls.